SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

# Eribulin · Days 1 and 8 of 21

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Previously treated advanced breast cancer or unresectable liposarcoma · NCCP 00228 · version 8 · 21-day cycle

| Patient name | DOB | Hospital no. | Date |
|---|---|---|---|
| | | | |

**Treatment context**
The aim is to control the cancer and its symptoms. The team reviews benefit, side effects and your priorities before continuing.

**Schedule**
Eribulin on Days 1 and 8 of each 21-day cycle. Day 15 is a planned treatment-free week. Treatment continues while beneficial and tolerated.

## 1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

## 2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

**Eribulin**
CHEMOTHERAPY

- ☐ Low blood counts, infection and sepsis
- ☐ Numbness, tingling or weakness, sometimes lasting
- ☐ Fatigue, hair loss, nausea and constipation
- ☐ Changes to heart rhythm / QT in susceptible people

Discuss the exact indication, expected benefit, alternatives, uncertainty, serious and lasting risks, contraception and fertility, venous access, monitoring and the urgent contact plan. EU dose labels use eribulin base. 1.23 mg/m² base corresponds to 1.4 mg/m² mesylate in some trial reports; these are not two alternative doses.

**Individual aim / benefit, alternatives, material risks and questions:**

## 3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

**Prototype only.** This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00228 v8.

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